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Healthy Start: Overview and Policy Considerations

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The Healthy Start program (42 U.S.C. §254c-8) aims to improve health outcomes before, during, and after pregnancy. The program funds projects in geographic areas with high infant mortality rates (IMRs) and other adverse perinatal outcomes (see **Figure 1** for recent trends in the national IMR compared with Healthy Start sites). Healthy Start emphasizes flexibility in meeting family- and community-specific needs and funds local organizations to provide tailored health and social services to women, their partners, infants, and children (up to 18 months). The program also funds the Healthy Start Technical Assistance and Support Center, which provides training, resources, and support to grantees.

Healthy Start has historically been administered by the Maternal and Child Health Bureau within the Health Resources and Services Administration (HRSA) at the Department of Health and Human Services (HHS); the extent to which the HHS restructuring plan might affect the program is unclear. Healthy Start is statutorily authorized through FY2025; however, the program's funding has been provided through appropriations laws, most recently through December 11, 2026.

Overview

Healthy Start was established in 1991 as part of a presidential initiative to decrease IMRs. The pilot program funded 15 community-based sites where IMRs were 1.5 to 2.0 times higher than the national average. Grantees were tasked with identifying and developing community-based approaches to reduce IMRs and improve the health and well-being of women, infants, children, and families; such services included case management, outreach, and health education, among others. The Children's Health Act of 2000 (P.L. 106-310) codified, and authorized appropriations through FY2005 for, the program. The Healthy Start Reauthorization Act of 2007 (P.L. 110-339) reauthorized appropriations through FY2013 and added additional grant requirements. The Coronavirus Aid, Relief, and Economic Security (CARES) Act (P.L. 116-136) reauthorized appropriations through FY2025 and added evaluation requirements (including a U.S. Government Accountability Office [GAO] report discussed below), among other amendments. Recent funding is presented in **Table 1**.

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Table 1. Healthy Start Funding

FY2020-FY2026

Fiscal Year	Appropriation (millions)	Number of Awards	Average Award (millions)
2020	$125.5	101	$1.12
2021	$128.0	101	$1.13
2022	$131.8	101	$1.05
2023	$145.0	111	$1.10
2024	$145.0	115	$1.08
2025	$145.0	115	$1.08
2026	$145.3	114	$1.08

Source: CRS review of appropriations laws. Totals reflect enacted amounts and may not account for agency reallocations or transfers. Award details were compiled using archived HRSA Congressional Budget Justifications (CJs), available at https://www.hrsa.gov/about/budget.

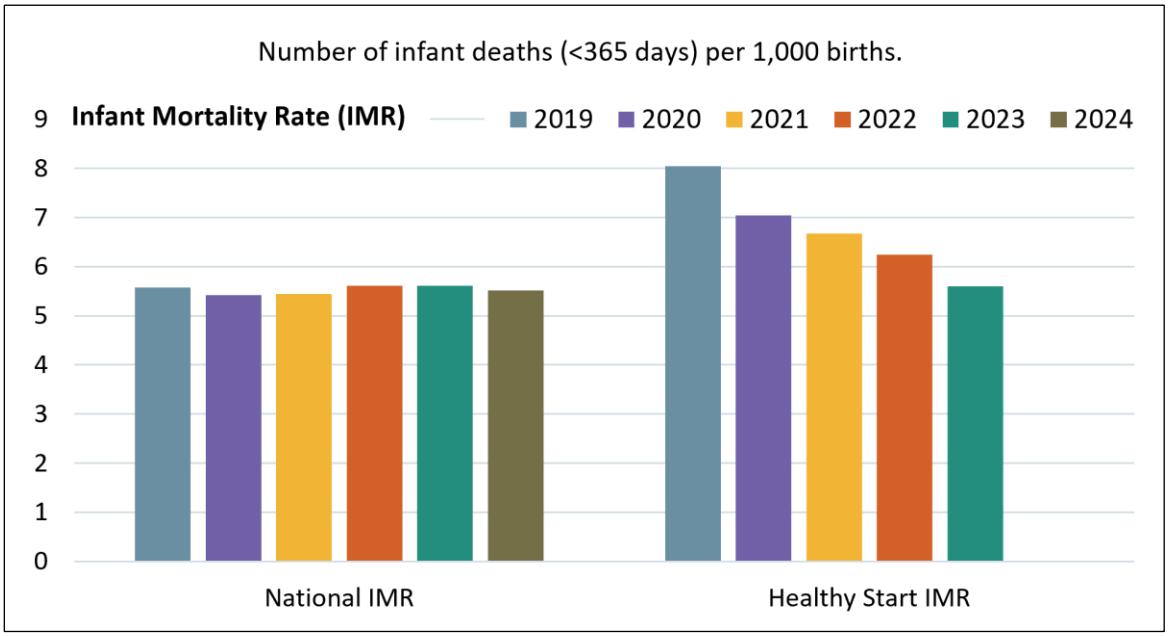
Notes: FY2025 and FY2026 totals reflect final amounts based on HRSA's FY2026 Operating Plan, available at the above link. Average award information for these years was provided to CRS by HRSA.

In FY2024, the program funded a cohort of 115 grantees across 37 states, the District of Columbia, and Puerto Rico. In addition to the services discussed above, grantees provide clinical services that may include prenatal, postnatal, or mental health care; screening and referral services; and peer support, provider training, and other social services. New to the FY2024 cohort, all grantees must offer group prenatal health and parenting education services. Each Healthy Start project has a Community Consortium composed of community members and is required to coordinate services with agencies administering the Title V State Maternal and Child Health Block Grant program.

Healthy Start has also provided supplemental grants aimed at training doulas, addressing infant health disparities through other determinants of health, and developing innovative care models. Congress has at times directed supplemental funding to Healthy Start to respond to emerging issues, such as childhood lead exposure during the Flint water crisis (P.L. 114-322; P.L. 114-254). Most recently, Healthy Start's pilot program to address high blood pressure during pregnancy received targeted FY2025 appropriations.

Figure 1. Infant Mortality Rate (IMR) Trends: National Rates vs. Healthy Start Rates, 2019-2023

Figure is interactive in HTML report version.



Source: Compiled by CRS using data provided by HRSA and the National Center for Health Statistics.

Notes: HRSA notes that higher IMRs for Healthy Start participants likely reflect the high-risk populations targeted by the program.

Program Evaluations and Policy Debates

Healthy Start has undergone multiple program evaluations since its inception. The most recent national evaluation in 2017 used a multimethod and multiyear approach to examine outcomes among Healthy Start participants compared with nonparticipants. Using program data, vital records, and other surveillance data, the evaluation indicated Healthy Start participants were more likely than nonparticipants to receive early and more frequent prenatal care, have lower rates of low birthweight infants, and implement safe sleep practices. However, many participants did not reach program targets related to breastfeeding and screenings for intimate partner violence. In 2024, a congressionally directed GAO report recommended that HRSA implement documented processes for reviewing program evaluation data and coordinating with other programs. According to GAO, HHS has partially addressed both recommendations. HRSA also initiated a national Healthy Start evaluation in 2021, which is expected to be published in late 2026.

Some observers assert that Healthy Start is duplicative with the Maternal, Infant, and Early Childhood Home Visiting program, while others contend that it is distinct, noting shared goals but differences in grantees, services provided, statutory authorities, and other requirements. The aforementioned 2024 GAO report also discussed similarities and differences in strategies, target populations, and services offered by related programs. Reflecting this debate, in recent years, some appropriations proposals have sought to eliminate funding for Healthy Start (e.g., H.Rept. 118-585; H.Rept. 119-271), while others have sought to continue, or in some cases expand, the program (e.g., S.Rept. 118-207; S.Rept. 119-55). The program was proposed for elimination in both the FY2026 and FY2027 presidential budget requests.

Policy Considerations

Two bills in the 119th Congress aim to reauthorize Healthy Start. H.R. 3302 would authorize $145 million in annual appropriations for each of FY2026 through FY2030; S. 3799 (as reported) would authorize the same total for each of FY2027 through FY2031. Aside from these proposals, Congress could reauthorize the program at the same, increased, or decreased funding levels; alter program requirements; or direct supplemental inquiries into specific health or program outcomes. Congress may also consider examining forthcoming evaluation data or recent IMR trends to understand program impact, take no action to continue the program, or pursue other considerations.

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